



WNHS PROCEDURE

Clinical Handover - WNHS Mental Health Service Outpatient and Consultation Liaison (OPCL)

Scope (Staff):	Mental health staff working in WNHS
Scope (Area):	Mental Health Service Outpatient and Consultation Liaison (OPCL)

This document should be read in conjunction with the [Disclaimer](#)

1. Aim

The purpose of this procedure is to ensure relevant, accurate and structured clinical handover occurs at the required times along the patient journey and supports safe patient care, consistent with the WA Department of Health (DoH) [Clinical Handover Policy](#). This includes on referral, initial assessment, treatment, throughout inpatient care and when transfer of care takes place. when transfer of care takes place.

2. Risk

Non-compliance with this policy will:

Breach legislative requirements	☐	Impact on Patient Quality of Care	☒
Breach National/State/Mandatory Policy	☒	Impact on Patient Safety	☒
Breach professional standards	☐	Impact financial operations	☐
Damage NMHS reputation	☐	Impact on Staff Safety	☐
Other	☐		

3. Definition

Clinical handover is the planned, structured provision of information to facilitate the transfer of professional responsibility and accountability for some, or all aspects of care for a patient, or group of patients, to another person or professional group on a temporary or permanent basis.

To ensure that all inpatient and outpatient clinical handovers maintain the standards as outlined in the definition above, this guideline applies to the following handover situations:

- WNHS Mental Health Service Outpatient and Consultation Liaison (OPCL) multidisciplinary team handovers
- transfer of patient care to another clinician
- handovers with other wards/departments of the hospital, of patients and their carers
- handover to external health care providers.

4. Guiding Principles

- Effective communication between health workers regarding patient care has been recognised as an essential element in best practice patient care and in the reduction of adverse events.
- Clinical handover occurs using a process by which information and responsibility is transferred, risk is identified, and continuity of care is facilitated.
- There is a requirement for clinical handover to follow a structured process and for this purpose the iSoBAR mnemonic is utilised (Identity, Situation, Observations, Background, Agreed plan, Read back).

iSoBAR: a handover “how to”

i	IDENTIFY	Introduce yourself and your patient
S	SITUATION	Why are you calling? Briefly state the problem
O	OBSERVATIONS	Recent vital signs and clinical assessment
B	BACKGROUND	Pertinent information related to the patient
A	AGREED PLAN	What needs to happen? Assessment of the situation
R	READ BACK	Clarify and check for shared understanding. Who is responsible for what and by when?



Identify



Situation



Observations



Background



Agreed plan



Read back

- Patients, carers, and family members are involved where appropriate in the clinical handover process.

- The referring service retains the responsibility for the patient until handover to the receiving service or practitioner or the patient decides on an alternative process.

5. Inpatient Handover

- Handover for all inpatients being referred to or seen by OPCL clinicians occurs at morning Triage meetings and ward rounds each weekday. Meetings led by triage clinicians are held at OPCL and attended by OPCL multidisciplinary clinicians on duty that day. Ward rounds are conducted by the inpatient triage clinician on inpatient wards and EC. Updated inpatient information is collated to provide ongoing clinical handover to treating clinicians. Handover is kept updated on a working document and presented at this meeting. This includes:
 - Information on new inpatient referrals and referral sources.
 - Review of treatment/care provided, discharge and follow up plans for inpatients seen by the service including dates and clinicians involved.
 - Handover regarding patients reviewed by clinicians the previous day for follow up by clinicians rostered on the following day. This includes handover from weekend on-call clinicians.
 - Updated information from the ward on in-patients being seen by OPCL.
 - A record of review of progress regarding ongoing higher risk patients of concern.
- Inpatient clinical handover information regarding assessment, progress, treatment planning, includes input from patients, carers, and family members, where appropriate.
- Handover information is conveyed to the triage officer for collation
 - By email to the dedicated triage email inbox (preferred method)
 - Directly verbally by the treating clinician
- New e-referrals are noted and responded to by the Triage officer. The referrer is informed of the acceptance or otherwise of the referral into OPCL.
- Handover information is recorded by the triage officer each day on the inpatient spreadsheet using the iSoBAR format and stored electronically.
- Clinical staff records all relevant patient information in the patient's medical record according to mandatory requirements. This provides an abiding record of relevant patient information for ongoing patient management and historical information that may be relevant for future patient care and management.
- All required information regarding patient referrals, assessment, diagnoses and treatment is recorded on PSOLIS.
- Written assessment summaries and letters are provided to referrers and relevant health care providers involved in the patient's care as appropriate.

- All relevant and required patient related information is filed according to the clerical order of filing for medical records in the patient's medical file. This includes Mental Health Assessment ([SMHMR902](#)) Mental Health Triage ([SMHMR900](#)) and Mental Health Risk Assessment and Management Plan([SMHMR905](#)) which are filed in the Outpatient section of the patient's medical file.

6. Outpatient Handover

- Outpatient referrals are reviewed weekdays at the daily patient triage intake meetings attended by the outpatient triage officer and the MDT of clinicians on duty that morning. Reviews and decisions regarding further triage and allocation of new referrals to OPCL services are recorded on the E-referral electronic system. This includes reviews of higher risk patients of concern.
- Decisions regarding patient care are recorded by triage clinicians for administrative staff to organise patient appointment allocations.
- Clinicians' assessments are recorded on mandated Statewide Standardised Clinical Documentation ([SMHMR902](#)). This includes assessment information and management plans and a record of discussion regarding this with patients (and carers where appropriate). This is stored according to correct order of filing in the patient's medical record. Liaison with key stakeholders (e.g. other departments within the hospital, general practitioners, psychiatrists/clinical psychologist in the private sector, community mental health services) and referral to other services or agencies for care and treatment is also recorded as part of the patient record.
- Written assessment summaries and letters are provided to referrers and relevant health care providers involved in the patient's care as appropriate. Copies are filed in the correspondence section of the patient's medical file.

7. Handover for After Hours Medical Officers, Registrars and Consultants

- Clinical handover is emailed to the on-call clinician by inpatient triage at the end of the day/Friday for weekend on-call. In addition, where required, verbal handover (face to face or by telephone) is provided by the treating clinician to the on-call clinician regarding any issues of concern or any presentations expected out of usual business hours.
- Handover regarding patients seen after hours is emailed by 08:30 the following day/Monday morning to the triage officer (using iSoBAR based content) with a copy to relevant treating clinicians.

8. Handover of Patient Care

Between Outpatient and Consultation Liaison Clinicians

- Clinical handover occurs when patient care is transferred within OPCL from one treating clinician to another or where an additional clinician becomes involved in the patient's care. Assessment reports (legible if handwritten) referrals, triage reports and other relevant documentation is available on the medical chart and PSOLIS to the clinician taking on the care of the patient. Additional verbal handover is also sought and provided where required. Where clinically relevant and appropriate, handover occurs with the patient, carers, and family members present.
- It is the responsibility of the clinician handing over, as well as that of the receiving clinician, to provide and obtain effective clinical handover and include and inform the patient of this process.

Patient External Referral

- Clinical handover occurs when patient care is transferred to an external service provider such as a GP or community mental health service provider.
- In referral to other Health Department mental health services, Mandatory Statewide Standardised Clinical Documentation (SSCD) is used and sent onto the next Health Care Provider ([SMHMR916- Care Transfer Summary Form](#))
- For other referrals, a discharge letter or Transfer of Care and Treatment Summary following iSoBAR principles is used.
- When a patient does not keep the planned follow-up arrangements there must be active consideration and management of risk and communication with the referrer.

Patient Internal Hospital Referral

- Clinical handover occurs when patients are referred to other Departments within the hospital system using e-referrals incorporating all iSoBAR components, by e-referral response and additional verbal liaison where required, medical file notes, emails (patient de identified), letters, or relevant referral forms.

Related policies

[Clinical Handover at WNHS Policy](#)

[Chief Psychiatrist's Standards for Clinical Care](#)

[Mental Health Act 2014](#)

[WA DoH Clinical Handover Policy](#)

Related WNHS procedures and guidelines

[Women's and Perinatal Mental Health Referral and Management Guideline](#)

Useful resources (including related forms)

[SSCD Resources](#)

Keywords	Clinical handover, psychological medicine, mental health, procedure				
Policy Sponsor	Head of Department – WNHS Mental Health Service Outpatient and Consultation Liaison				
Policy Author	Consultant Clinical Psychology Director – WNHS Mental Health Service Outpatient Consultation and Liaison				
Date First Issued:	01/07/2021	Last Reviewed:	June 2025	Review Date:	June 2028
Approved by:	Mental Health Clinical Practice and Outcomes Committee			Date:	23/06/2025
Endorsed by:	Mental Health Clinical Practice and Outcomes Committee			Date:	23/06/2025
NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☐  Std 2: Partnering with Consumers ☐  Std 3: Preventing and Controlling Healthcare Associated Infection ☐  Std 4: Medication Safety		☒  Std 5: Comprehensive Care ☒  Std 6: Communicating for Safety ☐  Std 7: Blood Management ☐  Std 8: Recognising and Responding to Acute Deterioration		
Chief Psychiatrist's Standards of Clinical Care	Transfer of Care				
Printed or personally saved electronic copies of this document are considered uncontrolled					

Version History

Version	Date	Version Notes
1.0	February 2007	Date First Issued. Regular review and update- dates unknown
2.0	June 2014	Title: Department of Psychological Medicine Delivery of Care Guidelines KEMH DC 24: Clinical Handover
3.0	June 2017	Title: Clinical Practice Guideline: Clinical Handover – Mental Health (merged version Department of Psychological Medicine and Mother and Baby Unit)
3.1	February 2020	Regular review and update
4.0	July 2021	Formatting change. Title: WNHS Procedure: Clinical Handover- Department of Psychological Medicine
5.0	June 2025	Three yearly review- minor amendments include:

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		• Minor formatting changes • Title change: WNHS Procedure: Clinical Handover – WNHS Mental Health Service Outpatient and Consultation Liaison (OPCL) to reflect departmental/service name change • Abbreviations to reflect new name • Updated hyperlinks, added relevant policies
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